Patient Group Direction: **Ipratropium nebules (anaphylaxis)**			
Version: **1.0**	Date of introduction: **18/02/2025**	▼Black Triangle Product: **NO**	Classification: **POM**

For the administration only of the above medicine(s) by Registered Practitioners employed by the Trust.

This patient group direction must be agreed to and signed by registered health care professionals involved in its use. The service lead or manager is responsible for holding the original signed copy in the clinical setting where the PGD will be used. An electronic copy can be found in the approved PGD database on the Trust intranet.

Developed / Reviewed by:		
Lead Author	**Name**: Ben Clayton **Signature**:	**Position**: Resuscitation Lead **Date**: 12/02/2026
Physician/Consultant*	**Name**: Ben Clayton **Signature**:	**Position**: Resuscitation Lead **Date**: 12/02/2026
Lead Nurse / Allied Health Professional/ Lead Practitioner	**Name**: Donna Seccombe **Signature**:	**Position**: Clinical Matron for Patient Safety **Date**: 18/02/2026
Pharmacist*	**Name**: James Leavy **Signature**:	**Position**: Medicines Information Pharmacist **Date**: 11/02/2026
ALL signature boxes above MUST be complete before submission to Medicine Management Group for approval.		
Organisational authorisation by MMG (Chair, or Deputy)	**Name**: Joe Maguire **Signature**:	**Position**: Deputy Chief Pharmacist **Date**: 18/02/2026 **PGD Expiry Date: Feb 2029** (no longer than 3 years)

*For PGDs covering systemic antibiotic use, a microbiologist and/or antimicrobial pharmacist must be included in its development.

Version	Summary of Changes
1.0 (Replaces N/A)	New PGD to support treatment of anaphylaxis.

Patient Group Direction: **Ipratropium nebules (anaphylaxis)**			
Version: **1.0**	Date of introduction: **18/02/2025**	▼Black Triangle Product: **NO**	Classification: **POM**

You must be authorised by name under the current version of this PGD before you attempt to work according to it.

1. Clinical Condition

Indication	Acute bronchospasm due to anaphylaxis
Inclusion criteria	All adults and children aged over 1 month
Exclusion criteria	• Hypersensitivity to ipratropium bromide or any other excipients in the nebuliser solution. • Hypersensitivity to atropine or its derivatives. • Received a dose of nebulised ipratropium in last 20 minutes.
Cautions / Need for further advice	• Patients with pre-existing urinary outflow tract obstruction (e.g. prostatic hyperplasia or bladder-outflow obstruction). • Paradoxical bronchospasm. • Patients with or susceptibility to narrow-angle glaucoma (care required to protect patient's eyes from nebulised drug). • See BNF interactions under 'antimuscarinics'. • Pregnancy - manufacturer advises only use if potential benefit outweighs the risk. • Breast feeding - manufacturer advises caution, as it is not known whether ipratropium is excreted into breast milk although it is unlikely that it would reach the infant in significant quantities.
Action if excluded	• If patient falls into exclusion criteria refer to senior doctor, if patient is at a non-acute site request an ambulance via 999
Action if patient declines treatment/refuses medication	• Document patient refusal in patient's records and action taken. • Await emergency assistance, as above

2. Description of Treatment

Method or route of administration	• Nebulisation driven by oxygen at 6 L/min • Repeat as per dosages stated below if necessary • Protect the patient's eyes from nebulised drug by use of appropriately fitting face mask • If dilution of ipratropium bromide nebuliser solution is necessary use only sterile 0.9% sodium chloride.
Dose	Child 1 month–5 years: 125–250 micrograms as required; max 1 mg/day Child 6–11 years: 250 micrograms as required: max 1mg/day

	Child 12 years or adult: 500 micrograms as required, max 2 mg/day For all age groups, a single dose is usually required, with ongoing treatment with nebulized salbutamol
Frequency	Maximum frequency of 20-30 minutes. Do not exceed maximum daily dosage.
Duration of treatment	As outlined in dosage section to alleviate symptoms.
Quantity to supply / administer	Administration only (no supply) Available as 250mcg/1ml and 500mcg/2ml, check dose used is appropriate for age (see dose section above).
Storage	• Do not store above 25°C. • Store in original package. • The ampoule should be opened immediately before use and any solution remaining after use should be discarded.

3. Patient Information

Advice and information to patient/carer, including side effects	Unusual or life-threatening reactions require immediate medical attention. Information to patient/carer: • Explain treatment and course of action • Potential side-effects (see below) **ANTIMUSCARINIC SIDE EFFECTS** • Common or very common: Constipation; cough; diarrhoea; dry mouth; gastro-intestinal motility disorder; headache; sinusitis • Uncommon: Angle-closure glaucoma; atrial fibrillation; blurred vision; dizziness; gastro-oesophageal reflux disease (in adults); mydriasis; nasopharyngitis; nausea; palpitation; paradoxical bronchospasm; pharyngitis; rash; tachycardia; throat irritation; urinary retention. **IPRATROPIUM SPECIFIC SIDE EFFECTS** • Laryngospasm; pharyngeal oedema; stomatitis; vomiting; ocular accommodation disorder.
Follow-up advice to be given to patient or carer	**FOLLOW UP:** • Patients must be assessed by a clinician with appropriate training in anaphylaxis management, who will be responsible for their ongoing care

4. Staff Characteristics

Registered Profession (tick those that apply) ^ Cannot administer or supply controlled drugs (schedule 1-5) * Nursing Associates not eligible to use PGDs	☐ Chiropodists & Podiatrists ☐ Dental hygienists^ ☐ Dental therapists^ ☐ Dieticians^ ☐ Midwives ☑ Nurses* ☐ Occupational Therapists ☐ Optometrists	☐ Orthoptists ☐ Orthotists & Prosthetists ☑ Paramedics ☐ Pharmacists ☐ Physiotherapists ☐ Radiographers ☐ Speech & Language Therapists^ ☐ Registered Pharmacy Technicians^
Additional Qualifications	Nil in addition to registration (as above)	
Specialist Competencies	• Working knowledge of Trust policies and guidelines. • Up-to-date mandatory training (including anaphylaxis) and ILS/ALS training.	
Additional Requirements	• Evidence of continuing professional development (and any training and competence relevant to this PGD). • Nurses: Working knowledge of relevant organisation protocols and policies, including Medicines Policy and associated Standard Operating Procedures Policy, and Consent Policy where appropriate. • Paramedics: Working knowledge of the Health and Care Professionals Council Standards of proficiency and other relevant codes of professional practice. • Peer review sessions • Medical feedback • Appraisal review.	

5. Record keeping, Referral Arrangements and Audit Trail

Specify method of recording supply /administration including audit trail	**Record Keeping**
	• Document allergies & other adverse drug reactions clearly in the patient health record and inform their GP and other relevant practitioners/patient/carer for further reporting & action if required.
	• Report any adverse drug reactions to the (MHRA) through the yellow card reporting system (www.mhra.gov.uk).
	• The following will be recorded in the patient's records:
	  ○ Date and time of administration.
	  ○ Details of medicine administered including name, strength, form, dose, route.
	  ○ Response of patient to treatment

- Name and signature of healthcare professional acting under the PGD to administer the medicine.
- Relevant information that was given to the patient/carer.
- Record that consent gained (or refused) – if consent refused record actions taken

Referral arrangements: emergency team or emergency ambulance

6. References used in the development of this PGD:

- BNF (British National Formulary) online - accessed 15/04/25
- Summary of Product Characteristics Ipratropium Bromide 250 micrograms/1ml Nebuliser Solution (Boehringer Ingelheim Limited) https://www.medicines.org.uk/emc/product/3818/smpc# [accessed: 15/04/2025]
- Scottish Intercollegiate Guidelines Network (2024) *SIGN 245 Asthma: diagnosis, monitoring and chronic asthma management* (BTS, NICE, SIGN) – accessed 22/01/2025 https://www.sign.ac.uk/media/2287/ng245-asthma-guideline-final-v11.pdf

Contacts for more information on the status of this document	
Royal Devon PGDs Pharmacy Administrative Office Pharmacy Department RD&E Hospital (Wonford) Tel: 01392 402441	
PGD Template Version: ***2025-1***	*Template approved by MMG (March 2025)*

PGD Record Keeping
Expired PGDs including staff signed authorisation sheet(s) should be scanned and retained on a shared dept/service Trust network drive for a period after the expiry date as follows: - 25 years if it relates to children - 8 years if it relates to adults only - 10 years if relates to use of an implant

Patient Group Direction: **Ipratropium nebules (anaphylaxis)**			
Version**: 1.0**	Date of introduction**: 18/02/2025**	▼Black Triangle Product: **NO**	Classification: **POM**

PGD's DO NOT REMOVE INHERENT PROFESSIONAL OBLIGATIONS OR ACCOUNTABILITY

It is the responsibility of each professional to practice only within the bounds of their own competence and in accordance with their own Code of Professional Conduct.

Note to authorising Managers: A paper copy of the full PGD must be kept in the relevant clinical area together with this sign-off sheet. Authorised staff can be signposted to the intranet where they can view the document.

CLINICAL AREA	LOCATION / DEPARTMENT

I have read and understood the Patient Group Direction and agree to supply / administer this medicine only in accordance with this PGD.

Name of Registered Professional	Signature	Authorising Manager	Date
			DD / MM / YYYY
			DD / MM / YYYY
			DD / MM / YYYY
			DD / MM / YYYY
			DD / MM / YYYY
			DD / MM / YYYY
			DD / MM / YYYY
			DD / MM / YYYY
			DD / MM / YYYY
			DD / MM / YYYY
			DD / MM / YYYY
			DD / MM / YYYY
			DD / MM / YYYY
			DD / MM / YYYY
			DD / MM / YYYY
			DD / MM / YYYY
			DD / MM / YYYY
			DD / MM / YYYY
			DD / MM / YYYY
			DD / MM / YYYY
			DD / MM / YYYY